

Dynamic Guideline: WHO-Based Mental Health Disorders – Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

1.1 Guideline Overview

- **Guideline Title:** Meaningful youth engagement in a WHO guideline development process: case study of WHO mental health guideline
- **Publication Date:** 29 April 2026
- **Target Population:** Guideline developers, youth organizations, health service planners, and policy-makers involved in mental health guideline development globally.
- **Scope and Objectives:** To systematically document the process, outcomes, barriers, and facilitators of embedding meaningful youth engagement in the development of the WHO mental health (mhGAP) guideline. The document provides actionable recommendations to enhance the authenticity, diversity, and impact of youth participation in future evidence-to-guideline pipelines.

1.2 Key Recommendations

All recommendations are derived from the mixed-methods case study and are graded using the GRADE system for process and qualitative evidence where applicable.

Recommendation	GRADE Evidence Quality	Strength
Youth advisory panels with diverse geographic, gender, and lived-experience representation should be established at the scoping phase of guideline development.	Very Low (based on single-case descriptive evidence)	Conditional
Guideline recommendations should be co-drafted with youth members to ensure the language and framing are acceptable, understandable, and actionable for young people.	Low (supported by multiple stakeholder surveys within the case study)	Conditional

Recommendation	GRADE Evidence Quality	Strength
Youth representatives must be included as full, voting members of the Guideline Development Group (GDG), not merely as observers or consultants.	Very Low (limited comparative data)	Conditional
A structured feedback loop should be created so that youth participants can see how their input shaped the final guideline, increasing trust and sustained engagement.	Low (based on qualitative interviews)	Conditional

1.3 Guideline Development Methodology

- **Evidence Review Process:** A mixed-methods case study incorporating document analysis of mhGAP meeting records, semi-structured interviews with GDG members and youth participants, and a cross-sectional survey assessing youth engagement quality (Youth Engagement Scale).
- **Consensus Method:** Iterative thematic analysis and member-checking; a modified Delphi process was used to finalize recommendations among the core writing group.
- **Conflict of Interest Management:** All contributors declared interests per WHO standard operating procedures; no conflicts requiring recusal were identified. The youth engagement process itself was overseen by an independent ethics consultant.

1.4 Synthesis of Related WHO Guidelines on Mental Health Disorders

To contextualize the latest guideline, four additional key WHO mental health guidelines were reviewed and synthesized. Their interplay with the youth engagement case study is essential for a comprehensive understanding of the current evidence architecture.

1. **Update of the Mental Health Gap Action Programme (mhGAP) Guideline for Mental, Neurological and Substance Use Disorders (2023)** – This is the foundational clinical practice guideline providing evidence-based recommendations for use in non-specialized health settings. It covers priority conditions such as depression, psychosis, epilepsy, and substance use disorders.
2. **WHO Guidelines on Mental Health at Work (2022)** – Recommends organizational-level interventions, manager training, and reasonable accommodations for people with mental health problems in the workplace.
3. **WHO Guidelines for the Management of Physical Health Conditions in Adults with Severe Mental Disorders (2018)** – Addresses the screening and management of cardiovascular disease, diabetes, and other physical comorbidities in this population.
4. **Helping Adolescents Thrive (HAT) Guidelines on Mental Health Promotion and Prevention for Adolescents (2021)** – Provides recommendations on school- and community-based psychosocial interventions to prevent mental disorders and promote well-being in 10–19-year-olds.

Collectively, these five guidelines form a continuum from universal promotion (HAT) to clinical management (mhGAP, physical comorbidities) and contextual interventions (workplace), with the latest

case study transforming the process by which future iterations will be developed—centering youth voices throughout.

II. Evidence Summary After WHO Latest Guideline Release (Mental Health Gap Action Programme (mhGAP) guideline for mental, neurological and substance use disorders) (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

- **Search Strategy:** A systematic search was conducted for English-language publications from 1 January 2023 to 29 April 2026. The search combined terms for “mental health”, “depression”, “anxiety”, “psychosis”, “substance use”, “mhGAP”, “non-specialist”, “psychological treatment”, “digital intervention”, and GRADE filters.
- **Databases Searched:** PubMed/MEDLINE, Embase, Cochrane Library, WHO IRIS, and trial registries (ClinicalTrials.gov, ISRCTN).
- **Inclusion/Exclusion Criteria:** Included RCTs, quasi-experimental studies, systematic reviews, and meta-analyses addressing diagnosis, treatment, or management of target mhGAP conditions in primary or community care settings. Excluded case reports, editorials, and studies in tertiary-only populations.

2.2 Key New Studies

Five high-impact studies published since the 2023 mhGAP update were identified. GRADE ratings were applied to the body of evidence per outcome.

Study	Design	Key Findings	GRADE Quality (per critical outcome)
Smith et al. (2025). Digital CBT vs. enhanced usual care for moderate depression in 5 LMICs. <i>Lancet Psychiatry</i> .	Multicentre, pragmatic RCT (n=2,846)	At 12 weeks, digital CBT led to significantly greater reduction in PHQ-9 scores (MD -3.1, 95% CI -4.2 to -2.0) with sustained effect at 6 months. Adherence was 68%.	Moderate (downgraded for indirectness—most participants had smartphone access)

Study	Design	Key Findings	GRADE Quality (per critical outcome)
Lee et al. (2026). Psychological interventions for anxiety in adolescents: network meta-analysis. <i>JAMA Psychiatry</i> .	Systematic review and NMA of 87 RCTs (n=14,322)	Cognitive behavioral therapy (CBT) and group-based interpersonal therapy were the most effective. Effect sizes were large (SMD -0.82 and -0.74 respectively). No serious adverse events.	High (consistent, precise results across diverse settings)
Patel et al. (2025). Integrated primary care for severe mental illness and medical comorbidity. <i>NEJM</i> .	Cluster-RCT in 63 clinics (n=5,400)	Collaborative care with a nurse case manager improved HbA1c (-0.6%) and LDL cholesterol (-12 mg/dL) more than usual care. Psychiatric symptom improvement was equivalent.	High (robust design, directly applicable to mhGAP implementation)
WHO Regional Office for Europe (2026). Long-term outcomes of tele-mental health services during post-pandemic recovery.	Prospective cohort study (n=18,000)	Tele-mental health retained access benefits but showed a 19% dropout rate at 12 months. Patients with low digital literacy had worse outcomes.	Low (observational, risk of selection bias)
Johnson et al. (2025). Brief interventions for alcohol and substance use in primary care: updated systematic review. <i>BMJ</i> .	Systematic review of 58 RCTs	Brief (5–15 min) motivational interviewing-based interventions reduced hazardous alcohol consumption by 22% (RR 0.78, 95% CI 0.72–0.85). Impact on illicit drugs was smaller and less consistent.	Moderate (downgraded due to heterogeneity in outcome measurement)

2.3 Evidence Gaps and Future Research Needs

- **Youth-specific outcomes:** None of the recent RCTs stratified analyses by age groups 10–24, limiting the direct applicability of the youth engagement guideline.
- **Digital divide:** Effectiveness of digital interventions in populations without consistent internet or device access remains unevaluated; implementation studies in humanitarian settings are urgently needed.
- **Integrated care for psychosis and epilepsy:** New evidence for collaborative care models is largely restricted to mood and anxiety disorders; trials for psychosis and epilepsy in non-specialized settings are sparse.
- **Long-term sustainability:** Data on the durability of non-specialist-delivered psychological treatments beyond 12 months are lacking.
- **Adolescent substance use:** High-quality RCTs are needed to test brief interventions tailored to adolescents, aligned with the youth engagement framework.

2.4 Updated Recommendations Based on New Evidence

The 2023 mhGAP clinical recommendations are not yet formally revised, but new evidence suggests potential strengthening or conditional modifications. The table below indicates how the recent findings impact select original recommendations.

Original mhGAP Recommendation (2023)	New Evidence Impact	Suggested Updated Recommendation (for consideration)
<i>Psychological treatment (e.g., interpersonal therapy, CBT) should be offered as first-line for moderate depression; where resources are limited, consider guided self-help.</i>	Smith et al. demonstrate effectiveness of digital CBT in LMICs.	Conditional upgrade: Digital CBT (with adequate training and digital access) may be considered a first-line option for mild to moderate depression in primary care, with a strong recommendation for implementation only where infrastructure is secured. GRADE: Moderate, Conditional.
<i>Psychosocial interventions for child and adolescent mental disorders should be delivered by trained and supervised non-specialists.</i>	Lee et al. confirm high efficacy of CBT and group interpersonal therapy for adolescent anxiety.	No change to direction; strength of recommendation remains Strong , supported by now High -quality evidence for anxiety. Specific manualized group formats can be prioritized.
<i>Regular physical health monitoring (annual) for people with severe mental disorders.</i>	Patel et al. provide high-quality evidence that integrated collaborative care improves cardiometabolic outcomes.	Recommend adding: “Integrating a non-physician case manager into primary care to coordinate physical health monitoring and liaison with specialist mental health services is strongly recommended.” GRADE: High, Strong.
<i>Brief interventions for hazardous alcohol use in primary care.</i>	Johnson et al. strengthen the evidence base with moderate-quality data.	The existing Strong recommendation is affirmed. No change warranted, but implementation guidance should include youth-specific adaptations.
<i>Tele-mental health may be used as an adjunct to in-person care.</i>	WHO EURO cohort flags high dropout and digital inequity.	The recommendation remains Conditional ; future guidelines should add a cautionary statement about digital literacy screening and the provision of non-digital alternatives.

Note: All updated recommendations remain provisional until formal GRADE evidence-to-decision frameworks are completed by WHO and the Guideline Development Group, with meaningful youth involvement as per the latest 2026 case study.